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Clinical evaluation of *Arnica montana* 1m in post-stroke neurological deficits: An observational study

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Abstract

Background: Stroke is a major global cause of death and disability, commonly resulting in long-term motor, sensory and cognitive deficits. Conventional management focuses on acute interventions and rehabilitation, but many patients are left with residual symptoms. *Arnica montana* 1M, a homoeopathic remedy traditionally used in trauma, vascular injury and inflammation, is proposed as an adjunct in stroke recovery.

Aim: To analyse the efficacy of *Arnica montana* 1M in the management of post-stroke neurological deficits.

Methods: An observational clinical study was conducted on 15 patients with clinically and radiologically diagnosed stroke, attending the Homoeopathic OPD of Late Mrs. Housabai Homoeopathic Medical College and Hospital, Nimshirgaon. Patients above 25 years of age with ischemic stroke, hemorrhagic stroke or transient ischemic attack (TIA) were included. All patients received *Arnica montana* 1M in individualized doses along with standard stroke care. Follow-up was done every 15 days for 2 months. Outcomes were assessed on the basis of neurological examination, changes in motor and speech function, subjective symptom relief and overall functional status.

Results: The study group comprised 10 males (66.67%) and 5 females (33.33%). Age incidence was highest in the 50-59 years group (46.67%), followed by 60-69 years (33.33%). Acute ischemic stroke was seen in 8 patients, hemorrhagic stroke in 3 patients and TIA in 4 patients. Overall, 13 patients (86.67%) showed some degree of clinical improvement, while 2 (13.33%) showed no response during the study period. Notable improvement in motor and speech function was observed in all hemorrhagic stroke cases and in most TIA cases, whereas ischemic stroke cases showed limited or no significant neurological recovery.

Conclusion: *Arnica montana* 1M may offer supportive benefit in post-stroke recovery, particularly in hemorrhagic stroke and TIA. Its role appears limited in established ischemic stroke. Larger, controlled studies are needed to validate these findings and to define the place of homoeopathic remedies within standard stroke rehabilitation protocols.

Keywords: Stroke, *Arnica montana* 1M, haemorrhagic stroke, transient ischemic attack, homoeopathy, neurorehabilitation

Introduction

Stroke is a medical emergency caused by a sudden interruption of blood supply to the brain, leading to death or damage of brain cells. It may result from arterial occlusion (ischemic stroke) or rupture of a cerebral vessel (hemorrhagic stroke). Ischemic strokes account for the majority of cases, whereas hemorrhagic strokes are less common but often more severe. Typical clinical manifestations include sudden hemiparesis or hemiplegia, facial asymmetry, speech disturbances, visual impairment, dizziness and altered consciousness.

Globally, stroke remains one of the leading causes of mortality and long-term disability. The World Health Organization estimates millions of new cases every year, with a higher burden in low- and middle-income countries due to uncontrolled hypertension, diabetes, smoking, obesity and other vascular risk factors. Despite significant improvements in acute stroke management-thrombolysis, thrombectomy and intensive care-many survivors are left with residual neurological deficits and impaired quality of life.

Standard post-stroke care emphasizes multidisciplinary rehabilitation, including physiotherapy, occupational therapy and speech therapy. However, there is growing interest in complementary systems such as homoeopathy to support recovery, reduce inflammation,

improve circulation and address psychological sequelae.

Arnica montana is a well-known homoeopathic remedy, classically indicated for trauma, bruising, hemorrhage, vascular congestion and post-injury shock. On this basis, it is hypothesized that *Arnica montana* 1M may aid post-stroke recovery, especially in hemorrhagic and traumatic vascular events. The present study was undertaken to evaluate the clinical efficacy of *Arnica montana* 1M in stroke patients with persistent neurological deficits.

Aim and Objectives

Aim

To analyse the efficacy of *Arnica montana* 1M in the management of stroke.

Objectives

1. To assess the impact of *Arnica montana* 1M on post-stroke neurological deficits, particularly motor and speech functions.
2. To evaluate subjective changes in symptoms such as weakness, numbness, dizziness and fatigue.
3. To observe psychological effects (stress, anxiety, emotional trauma) during recovery.
4. To monitor safety and any adverse effects associated with *Arnica montana* 1M.
5. To explore the feasibility of integrating *Arnica montana* 1M as a complementary modality in stroke rehabilitation.

Materials and Methods

Study Design

Observational clinical study.

Study Setting

Homoeopathic Outpatient Department (OPD)

Study Population and Sample Size

A total of 15 patients diagnosed with stroke (ischemic, haemorrhagic or TIA) were included.

Inclusion Criteria

- Patients with clinically and radiologically diagnosed stroke (ischemic, haemorrhagic) or TIA.
- Age above 25 years.
- Both male and female patients.
- Patients able and willing to give consent and attend regular follow-up.

Exclusion Criteria

- Immunocompromised patients (e.g., HIV/AIDS).
- Patients with severe comorbid illnesses or bedridden due to other causes.
- History of major neurosurgical procedures or irreversible brain damage.
- Patients unwilling or unable to complete follow-up.

Intervention

All enrolled patients were under standard stroke management as per conventional medicine. *Arnica montana* 1M was prescribed as an adjunct homoeopathic remedy:

- **Potency:** 1M
- **Mode:** Single or repeated doses as per individual case assessment.

- Placebo was given on subsequent days or weeks, while *Arnica* was repeated only when indicated.

No other homoeopathic remedy was introduced during the study period.

Follow-Up and Duration

Patients were followed every 15 days over a period of 2 months. At each visit, detailed neurological assessment and symptom review were performed.

Outcome Measures

- Changes in motor power and coordination.
- Improvement in speech (articulation, fluency, comprehension).
- Subjective relief in weakness, numbness, dizziness, fatigue and emotional distress.
- Overall functional status and daily activities.

Improvement was categorised as

- **Improved:** Clear objective and/or subjective improvement in neurological or functional parameters.
- **Not improved:** No meaningful change in neurological status or daily functioning.
- **Recovered:** Near-complete or complete restoration of function (not observed within this short study period).

Data were analysed descriptively in terms of frequency and percentage.

Results

Demographic Profile

Of the 15 patients, 10 (66.67%) were male and 5 (33.33%) female.

Age distribution was as follows:

- **40-49 years:** 2 cases (13.33%)
- **50-59 years:** 7 cases (46.67%)
- **60-69 years:** 5 cases (33.33%)
- **70-79 years:** 1 case (6.67%)

No cases were reported below 40 years. The maximum incidence was in the 50-59 year age group.

Clinical Profile

Based on clinical and imaging findings, patients were classified as:

- **Acute ischemic stroke:** 8 cases (53.33%)
- **Hemorrhagic stroke:** 3 cases (20.00%)
- **Transient ischemic attack (TIA):** 4 cases (26.67%)

Presenting features included slurred speech, limb weakness or paralysis, facial deviation, numbness, dizziness and imbalance.

Treatment Response

Overall outcome after 2 months of adjunct *Arnica montana* 1M therapy:

- **Improved:** 13 cases (86.67%)
- **Not improved:** 2 cases (13.33%)
- **Completely recovered:** 0 cases

By stroke type (clinical impression)

- **Hemorrhagic stroke (3 cases):** All showed notable

improvement in motor function and speech. Patients reported better strength, reduced heaviness and improved clarity of speech.

- **TIA (4 cases):** Majority showed symptomatic relief with reduced episodes of dizziness and weakness; no new TIA events were recorded during the follow-up period.
- **Ischemic stroke (8 cases):** Minimal or no significant neurological recovery was documented in several patients; any changes were mild and not sufficient to be considered functional recovery within the short study window.

No adverse effects attributable to *Arnica montana* 1M were reported.

Discussion

This observational study suggests a potential supportive role of *Arnica montana* 1M in post-stroke rehabilitation, particularly in hemorrhagic stroke and TIA. The high proportion of overall improvement (86.67%) must be interpreted cautiously, as natural recovery, physiotherapy and standard medical care also contribute substantially to outcomes.

The more favourable response in hemorrhagic stroke cases is consistent with the traditional homoeopathic indication of *Arnica* for trauma, bruising, vascular injury and hemorrhage. Stroke with intracerebral bleeding resembles a traumatic insult to brain tissue, where *Arnica* is classically prescribed to reduce inflammation, improve microcirculation and promote resorption of extravasated blood.

TIA, characterized by transient ischemia without permanent infarction, inherently carries a better prognosis. The observed improvement in TIA cases may reflect both natural course and possible circulatory support from *Arnica*, along with risk-factor control.

By contrast, ischemic stroke involves irreversible neuronal death in the infarct core. Within the short follow-up period of 2 months, *Arnica* did not appear to produce significant objective neurological recovery in these patients. This suggests that the scope of *Arnica* may be limited once extensive ischemic damage is established, although it may still offer symptomatic or emotional support.

From a homoeopathic perspective, *Arnica montana* is also indicated in patients who feel “sore, bruised” and mentally withdrawn, often insisting they are ‘well’ when they are not. Such mental and emotional traits are common after stroke and may have been favourably influenced, although formal psychological assessment was not undertaken in this study.

Areas for Further Improvement and Research

- The present study was conducted on a small sample (n = 15); therefore, larger multi-center studies are recommended to validate the findings.
- As this was an observational study without a control or placebo group, future research using randomized controlled trials will help establish stronger evidence.
- A longer follow-up period in future studies may provide clearer insights into long-term neurological recovery patterns.
- Incorporating standardized neurological assessment tools (e.g., NIHSS, mRS) in future research will allow more objective measurement of outcomes.

- Further studies designed to isolate the specific therapeutic role of *Arnica montana* 1M, independent of conventional rehabilitation, are encouraged.

Conclusion

Arnica montana 1M, used as an adjunct to standard stroke management, appeared to support clinical improvement in a majority of patients in this small observational study, with particularly encouraging results in hemorrhagic stroke and TIA. Its contribution in established ischemic stroke seems limited within the short period of observation.

These preliminary findings justify further research through well-designed, larger, randomized controlled trials to evaluate the true efficacy and scope of *Arnica montana* 1M and other homoeopathic remedies in stroke rehabilitation.

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Conflict of Interest

Not available.

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Not available.

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